



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : زينب احمد حسن عبدالعال
المحافظة : الجيزة
الحى / المدينة : مركز الجيزة
رقم البطاقة : 24907022101109 رقم التليفون : 01114465156
العمر (تاريخ الميلاد) : 02-07-1949 المهنة : ربه منزل
العنوان : الجيزة
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي
الاورام المنتشر [14456]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زينب احمد حسن عبدالعال National ID: 24907022101109	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

زینب
احمد حسن عبدالعال
قرية ناهيا
مركز كرداسة - الجيزة

٢٠٢١/١٠/٢٤ ٢٤٩.٧.٢٢١.١١.٩

١٩٠٢٦٠١٥٨

٢٠٢١/١٠ ٢٤٩.٧.٢٢١.١١.٩

بدون عمل

أنثى
مستلمة
متزوجة

حسين عبداللطيف حفاوي شحاته
البطاقة سارية حتى ٢٠٢٧/١٠/٢٤

١

Patient Name : الأستاذة / زينب احمد حسن عبدالعال	Request No. : 13C05099
Patient No. : 24907022101109	Request Date : 01/10/2024
Path No. : 3712/24	Verification Date : 16/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 75 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 30/9/2024.

Gross Examination:

- Multiple grayish white tissue cores measured 0.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
- P63: Negative.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- P63: Negative.
- ER: Positive (score 8 /8).
- PR: Positive (score 8 /8).
- Her2: Negative (score 0).

Verified by :

DR.Menna Nabil

Printing Date : 22/10/2024 11:10:50

Page 1 of 2

Printing By : Ali mostafa

Name: زينب احمد حسن عبد العال		
MR. : 3065		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 75-year-old individual with bilateral breast cancer described as luminal NAET. Bilateral modified radical mastectomy (MRM) was performed in September 2025, followed by placement of a port. Adjuvant therapy was initiated with letrozole (Femara) and zoledronic acid (Zomeata) for a planned duration of six months. Subsequent management included continuation of letrozole (Femara) together with calcium supplementation (1 tablet daily, 90 mg) and a single dose of alpha-interferon for breast imaging follow-up, including ultrasound and chest X-ray, as indicated. The patient returned for letrozole administration and was placed on supportive treatment for three months. Bilateral sub-muscular (SM) implants placed in May 2026 were noted, and the patient was scheduled for a follow-up clinic visit after three months. The patient now reports severe knee joint pain consistent with marked osteoarthritis. A bone scan was ordered, and a Dexa scan was arranged. The patient was referred to the pain clinic for further management. Letrozole (Femara) and supportive treatment were continued for an additional three-month period. The current plan is to monitor pain control, assess bone health, and continue endocrine therapy as prescribed.		
تحريراً في ,		
	التاريخ : 19-09-2026	الوقت :
Reham Ahmed Fathy Ibrahim		الطبيب المعالج :
يعتمد ,		

Patient Name	زينب احمد حسن	Age	75
Sex	Female	Special habits	
Family History		Comorbidity	Hypertensive
Surgical History		Telephone	01114465156
Menopausal Status	Post menopausal	Marital Status	
ID	24907022101109	Medical ID	3028/2024
Date	8/10/2025	Reason of MDT	Post Op

Medical Report

Pathology: IDC bilateral

RT: ER: 8/8. PR: 8/8. HER-2: 0. Ki67: 10%

LT: ER: 8/8. PR: 8/8. HER-2: 0. Ki67: 10%

T4 N0 M0

S.M:

Metsworkup:

- CT: free

Recommendations :

- Patient is on hormonal therapy
- Bilateral SM + axillary sampling
 - Rt: Residual IDC grade 2 (0/9)
 - Lt: Residual IDC grade 1 (0/8)

MDT DECISION

For: bilateral PORT
Hormonal tt AI (femara) + zometa/ 6months

Name	د/ ندي حسن	د/ مني زكي	
Signature			



Patient Name: Zainab Ahmed Hassan Abd Al Aal	Date : 08/09/2024
Exam : C.T CHEST	Age : 75 Year(s)
Medical ID: 328/2024	Pat. ID: 78841

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Right breast UIQ parenchymal speculated soft tissue lesion measures about 1.9 x 1.9 cm is seen inseparable from the pectoral muscles.
- Left breast LOQ parenchymal speculated soft tissue lesion measures about 1.7 x 2.1 cm is seen.
- Subcentimetric bilateral level I axillary lymph nodes.
- No sizable suspicious pulmonary nodules or masses.
- Mild bilateral upper lobes bronchectatic changes.
- Middle lobe, lingula and bilateral lower lobes atelectatic bands.
- Subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Bilateral renal small stones are seen, largest is seen at left renal pelvis measures about 6 mm with mean density about 600 HU. No back pressure changes.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.

Name: Mrs. Zelnab Ahmed Hassan

Date: 27/7/2024

ID:1124020837

Acc.:291164409

Referring Prof. Dr. Waleed Ibrahim El-Shemy

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts, revealed:

Indication: left breast palpable lump

Mammography & Tomosynthesis:

- Scattered fibroglandular densities (ACR-b).
- **Left breast:** upper outer quadrant posterior third: irregular shaped spiculated margin heterogeneous dense lesion with internal grouped / clustered pleomorphic microcalcifications measuring 3.7 x 3 x 5.3 cm and 7 cm distance from the nipple
- **Right breast:** no evidences of dense spiculated masses or architecture distortion. No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour of both sides.
- Normal areola / nipple complexes on both sides.

Ultrasonographic examination showed:

- Heterogeneous fibroglandular breast parenchyma
- **Left breast:**
 - ✓ Upper outer quadrant 2 o'clock irregular shaped spiculated margin heterogeneous hypoechoic solid lesion (corresponding to the lesion in mammography and palpable lump) 0.5 cm deep to skin, with posterior acoustic shadowing and internal vascularity of arterial waveform pattern
 - ✓ No gross evidence of pathologic axillary lymph nodes
- **Right breast:** No gross evidence of cystic or suspicious solid masses.
- No retro-areolar dilated ducts noted.
- Normal skin thickness and contour.
- Normal nipple/areola complexes.
- Bilateral non-specific axillary lymph nodes with preserved oval shaped and central echogenic hilum.

Patient Name : الأستاذ / زينب احمد حسن عبدالعال
Patient No. : 24907022101109
Path No. : 2776/25
Sex : Female
Age : 76 Y
Referring Doc :

Request No. : 13007668
Request Date : 08/09/2025
Verification Date : 07/10/2025
Financial Category : جوسف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Bilateral breast cancer, Post-NACT.

Nature Specimen:

- 1- Right simple mastectomy with sentinel lymph nodes.
- 2- Left simple mastectomy with sentinel lymph nodes.

Gross Examination:

1- Right breast:

- * Right simple mastectomy specimen measured 23x18x4 cm, with covering skin ellipse measured 18x10 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 3x2x1.5 cm, located 0.7 cm from the deep margin.
- * Attached axillary fat measured 5x4 cm, entangling 6 lymph nodes, the largest was 1.5x1x0.5 cm.
- * Sentinel lymph nodes (Intra-operative): Multiple fibrofatty tissue pieces collectively measured 4x4 cm, entangling 3 lymph nodes, the largest was 1x1x0.5 cm.
- * Total nodal count is: Nine lymph nodes.

1- Left breast:

- * Left simple mastectomy specimen measured 21x18x4 cm, with covering skin ellipse measured 17x10 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 3x2.5x1.5 cm, located 2 cm from the deep margin.
- * Attached axillary fat measured 4x4 cm, entangling 5 lymph nodes, the largest was 2x1x0.5 cm.
- * Sentinel lymph nodes (Intra-operative): Fibrofatty tissue piece measured 4x3 cm, entangling 3 lymph nodes, the largest was 1.5x1x0.5 cm.
- * Total nodal count is: Eight lymph nodes.

Verified by :

Dr.Duaa Mohamed

Printing Date : 07/10/2025 09:20:00

Page 1 of 4

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / زينب احمد حسن عبدالعال	Request No. : 13007668
Patient No. : 24907022101109	Request Date : 08/09/2025
Path No. : 2776/25	Verification Date : 07/10/2025
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 76 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

16 Blocks:
- Right; 1:3- TBS., 4- Surrounding breast., 5- N&A., 6- Deep margin., 7- SLNs(inta-operative).,
8:9- Rest of L.Ns.
- Left; 10:12- TBS., 13- N&A., 14- Deep margin., 15- SLNs., 16- Rest of L.Ns.

Frozen (Intra-operative) Results:
- Right breast; Free three sentinel lymph nodes (0/3).
- Left breast; Free three sentinel lymph nodes (0/3).

Microscopic Examination:
1- Right breast:
* Histological type: Residual viable invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- Actual tumor size: 3x2 cm.
- Tumor cellularity: 15%.
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* Microcalcification: Present.
* Nipple & areola: Free.
* Deep margin: Free.
* Sentinel lymph nodes (Intra-operative): Free three lymph nodes (0/3).
* Attached lymph nodes: Free six lymph nodes (0/6).
* Total nodal status: Free nine lymph nodes (0/9).
* Histologic response to therapy: Pathological partial response to therapy.

2- Left breast:
* Histological type: Residual viable invasive duct carcinoma.

Verified by :
Dr.Duaa Mohamed

Printing By : Mrs.Mayada Hegazy

Printing Date : 07/10/2025 09:20:00

Page 2 of 4



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / زينب احمد حسن عبدالعال	Request No.	: 13007668
Patient No.	: 24907022101109	Request Date	: 08/09/2025
Path No.	: 2776/25	Verification Date	: 07/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 76 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- Actual tumor size: 3x2.5 cm.
- Tumor cellularity: 60%.
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes (Intra-operative): Free three lymph nodes (0/3).
- * Attaches lymph nodes: Free five lymph nodes (0/5).
- * Total nodal status: Free eight lymph nodes (0/8).
- * Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:

Bilateral breast cancer, Post-NACT:

1- Right simple mastectomy with sentinel lymph nodes:

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 15%) (Actual tumor size 3x2 cm).
- DCIS: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Total lymph nodes: Nine free lymph nodes (0/9).
- * Stage: ypT2, ypN0.
- * RCB: 1.747, RCB class-II .

2- Left simple mastectomy with sentinel lymph nodes:

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 60%) (Actual tumor size 3x2.5 cm).

Verified by :

Dr.Duaa Mohamed

Printing Date : 07/10/2025 09:20:00

Page 3 of 4

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / زينب احمد حسن عبدالعال
Patient No. : 24907022101109
Path No. : 2776/25
Sex : Female
Age : 76 Y
Referring Doc :

Request No. : 13007668
Request Date : 08/09/2025
Verification Date : 07/10/2025
Financial Category : نفقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- DCIS: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Total lymph nodes: Eight free lymph nodes (0/8).
- * Stage: ypT2, ypN0.
- * RCB: 2.253, RCB class-II.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing By : Mrs.Mayada Hegazy

Printing Date : 07/10/2025 09:20:00

Page 4 of 4



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt